Prior Authorization Request Packet

Case pa_bf064149 · Aetna · MRI knee (lower extremity joint) without contrast (73721) · Policy Aetna CPB 0171
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1. Letter of medical necessity

October 11, 2026

Aetna Prior Authorization Department

RE: Marcus Lindqvist, DOB 1991-06-23, Member ID AET-513980027
Requested service: MRI knee (lower extremity joint) without contrast, CPT 73721
Ordering physician: Dr. Priya Raman, NPI 1891234567

To Whom It May Concern:

I am writing on behalf of Dr. Priya Raman to request authorization for a right knee MRI without contrast for Mr. Lindqvist, described in the clinic note as a "35-year-old male recreational soccer player". He has "persistent right knee pain and intermittent swelling". The note records: "He recalls a twisting injury during a game earlier this summer." On examination the note documents: "Moderate right knee effusion." It also documents "Medial joint line tenderness." and "McMurray test positive medially with a palpable click." The note further records "Lachman negative, posterior drawer negative." and "Range of motion 5 to 120 degrees, limited by pain." The assessment is: "Suspected right medial meniscus tear with persistent effusion and instability."

The following reviews this request against Aetna Clinical Policy Bulletin 0171 (https://www.aetna.com/cpb/medical/data/100_199/0171.html).

Criterion 1, persistent knee pain, swelling, and/or instability: Met. The note states: "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

Criterion 2, no response to at least 3 weeks of conservative therapy: Met. The clinic note states: "He has been doing physical therapy and taking ibuprofen, and has been using a brace, but the symptoms have persisted." The note does not give the duration of therapy. The duration comes from a staff-entered answer in the record, which states: "Yes. The patient completed about eight weeks of formal physical therapy, twice weekly, from early July through the end of August, plus daily NSAIDs." This is about eight weeks against the three required. This duration is not in the clinic note itself.

Criterion 3, multi-view x-rays ruling out fracture or loose body: Met. The note records: "Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

The policy also lists other indications for knee MRI: persistent true locking, suspected osteomyelitis

or septic joint, and evaluation of a knee tumor. None of these is documented, and this request does not rely on them. The note states: "He denies true locking of the knee." Regarding infection, the note records "No fever, no redness." and "No warmth or erythema." The record contains no information about a tumor.

In conclusion, the record documents all three criteria of the policy's pathway: persistent symptoms, failure of conservative therapy beyond three weeks, and x-rays excluding fracture and loose body. The documented therapy duration comes from the staff-entered response noted above. The note also states: "Continue physical therapy pending imaging." We respectfully request approval of CPT 73721 to evaluate the suspected medial meniscus tear.

Sincerely,

Dr. Priya Raman
NPI 1891234567

2. Criteria checklist with verified evidence

MET aetna-73721-c1: Persistent knee pain, swelling, and/or instability (giving way).
"Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

MET aetna-73721-c2: Not responding to at least 3 weeks of conservative therapy (conservative therapy consists of a combination of rest, ice, compression, elevation, plus NSAIDs and/or physical therapy).
"The patient completed about eight weeks of formal physical therapy, twice weekly, from early July through the end of August, plus daily NSAIDs." [manual]
code check: 8.0 weeks documented vs 3 required

MET aetna-73721-c3: Multi-view x-rays have been obtained and have ruled out a fracture or loose body in the knee.
"Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

MISSING aetna-73721-r1: Persistent true locking of the knee indicative of a torn meniscus or loose body (more than a momentary locking of the joint with the knee in a flexed position).

MISSING aetna-73721-r2: Suspected bone infection (i.e., osteomyelitis) or septic joint.

MISSING aetna-73721-r3: Detection, staging, and post-treatment evaluation of tumor of the knee.

3. Source visit note

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CHIEF COMPLAINT: Right knee pain and swelling.

HISTORY OF PRESENT ILLNESS: 35-year-old male recreational soccer player presents with
persistent right knee pain and intermittent swelling. He recalls a twisting injury during a
game earlier this summer. Since then he has had medial knee pain, swelling after activity,
and a sensation of the knee giving way on stairs. He denies true locking of the knee. No
fever, no redness. He has been doing physical therapy and taking ibuprofen, and has been
using a brace, but the symptoms have persisted.

PHYSICAL EXAM: Moderate right knee effusion. Medial joint line tenderness. McMurray test
positive medially with a palpable click. Lachman negative, posterior drawer negative. Range
of motion 5 to 120 degrees, limited by pain. No warmth or erythema.

IMAGING: Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint
spaces preserved, small effusion.

ASSESSMENT: Suspected right medial meniscus tear with persistent effusion and instability.

PLAN: MRI right knee without contrast (CPT 73721) to evaluate meniscus and ligaments.
Continue physical therapy pending imaging.
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